RIVERSIDE REGIONAL MEDICAL CENTER

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PATIENT NAME:	John Doe	**DOB:**	01/15/1984 (42yo, Male)	**MRN:**	#RMC-984210
ENCOUNTER DATE:	10/12/2026	**PLAN:**	United Healthcare	**STATUS:**	Outpatient

REQUESTED PROCEDURE: **CT Scan of Head with Contrast (CPT 70460) | ICD-10: G44.89**

CHIEF COMPLAINT:

Severe left-sided throbbing headaches persisting for 12 weeks with intermittent blurred vision and focal dizziness.

HISTORY OF PRESENT ILLNESS (HPI):

42-year-old male presents with persistent daily headaches lasting 12 weeks. Pain is localized to the left temporo-parietal region, rated 7-8/10. Headaches are atypical, awakening patient from sleep with associated nausea and visual disturbances.

PRIOR CONSERVATIVE MANAGEMENT:

Trial of conservative treatment attempted including outpatient physical therapy, ergonomic adjustments, and scheduled oral NSAIDs without clinical improvement. Symptoms remain refractory to non-pharmacologic and medical conservative measures.

PHYSICAL & NEUROLOGICAL EXAMINATION:

Vitals: BP 124/82, HR 72, SpO2 99% RA. Neurological examination abnormal: Cranial nerves II-XII intact except for subtle left-sided pronator drift on outstretched motor testing. Deep tendon reflexes asymmetric (2+ on right, 3+ on left brachioradialis). Coordination: mild dysmetria on left finger-to-nose testing. Sensation intact.

CLINICAL IMPRESSION & ASSESSMENT:

1. Intractable atypical chronic cephalalgia with focal neurological findings.
2. Rule out intracranial pathology, space-occupying lesion, or intracranial neoplasm.

ORDER & CLINICAL RATIONALE:

CT Scan of Head with Contrast (CPT 70460). Intravenous contrast clinically indicated for further diagnostic evaluation and evaluation of suspected mass given focal motor asymmetry. Patient has no history of contrast allergy or anaphylaxis.

Electronically Signed by: Dr. Sarah Jenkins, MD, Board Certified Neurologist **Date:** 10/12/2026 14:32 PST